MOOD-06 | Therapist only - all age groups

Maintaining progress and coping with recurring low mood

1 | Purpose, fit and boundaries

Purpose of this unit

Summarize what helped, identify personal signs of changes in functioning, and develop a flexible support-seeking plan. The result is a plan reviewed with the therapist, not a promise to prevent future low mood.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Do not diagnose relapse from a list of signs or wait for forms to be completed when help is needed. Do not change medication or treatment. Acute distress or safety concerns require appropriate assessment, without relying on this plan alone.

Reusing existing material

Core-08 provides a general maintenance framework. Here we choose mood-related signs and tools from MOOD-01 to MOOD-05 and consider when to seek professional help. Do not duplicate an existing support list.

> Draft for administrator review. Clinical review and final rights approval are still pending.

Mindful Path

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Working together with the worksheet

2 | A conversation about one situation

Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

1 | Question for discussion

What did I learn about situations, tools and conditions that support me?

2 | Question for discussion

What personal changes in functioning need attention?

3 | Question for discussion

What could I do first without overloading myself?

4 | Question for discussion

Whom can I approach, and what practical help will I ask for?

5 | Question for discussion

What will we agree with the therapist about follow-up and further contact?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

Prepare a simple way to tell an appropriate adult; recognizing and responding to distress is not the child's responsibility.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

Agree ways to seek help and confidentiality limits within treatment. Do not require constant monitoring or a promise to report every feeling.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

Include contact details only in authorized treatment records. Agree with the person when self-help is not enough.

OA | Older adults

Explore available ways to seek help and preferences, without assuming family is available or supervision is needed.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

Decide together whether practice between sessions is appropriate at all. Define one opportunity, boundaries, and desired support. Practice may take place only in the session. There is no required daily sequence or requirement to share the sheet with others.

At the next session

Check that the plan is practical and support is available. Returning difficulties do not erase learning; follow-up and treatment depend on clinical need.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

www.nice.org.uk/guidance/ng222/chapter/Recommendations
www.nice.org.uk/guidance/ng134/chapter/Recommendations

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